

ONCOLOGY PROGRESS NOTE

DATE: 28-Oct-2022

PATIENT: Robinson, Karen (MRN: AML021)

SUBJECTIVE: Patient seen in clinic today. She reports profound fatigue and new-onset bone pain in her lower back. She has been on Cycle 5 of Decitabine.

OBJECTIVE: Labs reveal rising blast count in peripheral blood (15%). Platelets 28K, ANC 0.4.

ASSESSMENT: Relapsed/Refractory adverse-risk (TP53 mutated) AML. Disease has progressed through first-line hypomethylating agent therapy (started on 25-Jun-2022). Prognosis is extremely poor. Options including clinical trials (none available locally for which she is a candidate) versus transition to comfort-focused care were discussed at length with patient and her daughter. Patient clearly expresses a desire to avoid further intensive chemotherapy and focus on quality of life.

PLAN:

1. Discontinue Decitabine.
 2. Admit to hospital for management of symptomatic pancytopenia and pain control.
 3. Consult Palliative Care service.
 4. Goals of care to be formally established as comfort-focused.
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PALLIATIVE CARE CONSULTATION

DATE: 30-Oct-2022

REASON FOR CONSULT: Goals of care discussion and symptom management for a 73 y/o F with end-stage AML (date of diagnosis: 14-Jun-2022)

DISCUSSION: Met with Ms. Robinson and family. She understands her prognosis is limited to weeks. She wishes to be kept comfortable and prioritize time with family. She does not desire CPR or mechanical ventilation (DNR/DNI status confirmed). Her primary symptom burdens are fatigue and a constant, aching sacral pain (rated 7/10). She also has significant anxiety.

PLAN:

1. **Pain:** Start scheduled long-acting Morphine (MS Contin 15 mg PO BID) with immediate-release Morphine 5mg PO q4h PRN for breakthrough pain.
 2. **Anxiety:** Start Lorazepam 0.5 mg PO TID PRN.
 3. **Transfusions:** Continue pRBC and platelet transfusions for comfort (e.g., to alleviate dyspnea or bleeding) but not to target specific numbers.
 4. **Disposition:** Family wishes to explore options for home hospice care. Will engage social work.
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FAX TO PRIMARY CARE PHYSICIAN

TO: Dr. Melissa Hart, MD

FROM: St. Jude's Hospital, Hematology Service

DATE: 28-Nov-2022

RE: DEATH OF PATIENT - Karen Robinson, DOB 13-Apr-1949

Dear Dr. Hart,

This is to formally notify you that your patient, Ms. Karen Robinson, passed away peacefully on our inpatient palliative care unit on **November 25, 2022**.

As you know, she had an aggressive, TP53-mutated AML and progressed rapidly. Her final days were focused on comfort, and she was surrounded by her family.

Cause of Death: Complications of Acute Myeloid Leukemia.

Sincerely,
Dr. Eleanor Vance
Oncology Service